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Accredited Continuing Medical Education Delivers: Evidence of Value, Trust, and Impact Across the Healthcare System

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ABSTRACT

Accredited continuing medical education and continuing professional development (CME/CPD) form one of the world's most extensive systems for improving healthcare quality, safety, and efficiency. In the United States, some 1,550 accredited providers deliver over 230,000 educational activities each year under the oversight of the Accreditation Council for Continuing Medical Education (ACCME). Drawing on recent ACCME data, findings from the 2025 CPD/CE Workforce Survey, and outcomes from the Menu of Criteria for Accreditation with Commendation, this article presents evidence that accredited CME/CPD delivers measurable value for clinicians, organizations, and populations. Beyond enhancing knowledge and skills, accredited CME/CPD improves clinician performance, strengthens interprofessional teamwork, advances adoption of new science, and supports organizational goals for quality, safety, and cost-effectiveness. Accredited CME/CPD operates through deliberate instructional design, continuous outcomes measurement, and a professional workforce that integrates learning with improvement. When empowered by institutional leadership, CME/CPD professionals become strategic partners in achieving system-wide change. Accreditation standards ensure independence from commercial influence and foster public trust in the integrity of medical education. Programs recognized with commendation demonstrate quantifiable improvements in healthcare quality, patient outcomes, and community health. Far from a compliance requirement, accredited CME/CPD represents an ethical, scalable infrastructure that translates evidence into practice and sustains improvement across the healthcare system. By investing in accredited education, leaders reinforce workforce capability, teamwork, and patient safety - demonstrating that education, when structured and protected by accreditation, is a central driver of healthcare value and trust.

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Introduction: CME/CPD as a Catalyst for Healthcare Value

Continuing medical education and continuing professional development (CME/CPD) constitute one of the most extensive and effective systems for improving healthcare performance and patient safety. Within the United States, the Accreditation Council for Continuing Medical Education (ACCME) oversees a system of accredited providers that produce more than 230,000 accredited educational activities annually for the nation's one million physicians [1]. Accredited providers include academic centres, professional societies, hospitals, and other entities that meet criteria emphasising independence from commercial influence, educational quality, and outcomes orientation. Through rigorous standards, verified outcomes, and a skilled professional workforce, accredited CME provides a scalable mechanism for improving care quality, efficiency, engagement, and well-being.

The accredited CME/CPD enterprise extends well beyond traditional lectures and large national conferences to include performance-improvement initiatives, simulation, point-of-care learning, interprofessional team training, case-based and problem-based formats, longitudinal coaching, quality/safety collaboratives, and workflow-embedded learning. This breadth reflects how clinicians learn and supports translation of evidence into care processes [2–4].

Health care leaders increasingly recognise that education is not simply a regulatory requirement but a strategic resource; an underused and low-cost solution capable of improving clinical performance, fostering effective teams, strengthening morale, and reducing burnout. Engaging clinicians in their own learning journey is an investment in human capital and institutional culture. Accredited CME ensures that this education is relevant, evidence-based, and independent,

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while aligning professional learning with system goals for better health, better care, and lower cost.

Accreditation systems worldwide share common aims including educational quality, independence, and impact, while differing in structure and implementation. Recent work has summarised national and regional approaches to CME/CPD and highlighted areas of convergence (e.g. standards, outcomes focus) [5–7]. The World Federation for Medical Education (WFME) has established a formal recognition systems for global high-quality CPD accrediting bodies including ACCME [8].

The purpose of this article is to present evidence that accredited CME delivers measurable value to clinicians, organisations, and communities, and to describe how leadership investment in education amplifies this value across the healthcare system. In doing so, this article also situates accredited CME as the operational infrastructure through which evidence, quality improvement, and interprofessional collaboration are translated into better care. Accredited CME is increasingly viewed by health system executives as the connective tissue that links clinical excellence with workforce sustainability, ensuring that improvement efforts are informed by learning science rather than compliance alone.

A Thriving and Evolving Educational System

The accredited CME enterprise continues to expand in scope, financial resilience, and alignment with organisational strategy. The 2024 ACCME Data Report describes a system that is both stable and dynamic, with total reported income of $3.7 billion in 2024, reflecting ongoing institutional investment in education that advances clinician performance and patient care. While commercial support declined modestly (5% year over year), registration fees accounted for 55% of total income and advertising and exhibit revenue exceeded $725 million, an 8% increase [1]. Only 7% of all educational activities receive any support from industry, a falling figure that represents increasing separation of accredited CME from the activities of the pharmaceutical industry and device manufacturers. This diversification illustrates a maturing enterprise that sustains innovation while maintaining independence from industry or from any single funding source. This financial and structural maturity has positioned CME as part of the essential learning infrastructure of health systems, supporting rapid dissemination of clinical innovations, policy updates, and safety practices within the workplace and among clinical teams.

Consolidation within health systems has resulted in fewer but more productive accredited providers, while educational formats continue to diversify. Enduring materials and live meetings remain dominant, and case conferences embed continuous learning and quality improvement within daily clinical practice. Together these formats serve complementary functions: enduring materials provide flexible access to specialised content, live meetings foster professional identity and collaboration, and case conferences sustain team-based improvement through longitudinal participation.

The workforce supporting this system is experienced, stable, and growing. Preliminary findings from the 2025 CPD/CE Workforce Survey, conducted by ACCME and collaborating organisations, show that among 524 respondents, they reported net continuing growth in staff and recruitment, that most program leaders have more than 15 years of experience, and 86% plan to remain in the field. These data depict a capable and committed workforce of approximately 6,000 individuals in the United States that underpins the reliability and success of the accredited CME system [9]. The depth and stability of this workforce enable consistent implementation of new science and improvement initiatives, helping ensure equitable and consistent access to advances in care across settings.

CME/CPD Improves Healthcare Quality, Safety, and Effectiveness

Extensive evidence demonstrates that accredited CME improves clinician performance and patient outcomes. A synthesis of 39 systematic reviews identified CME as one of the most effective methods for translating knowledge into improved practice [10]. Within the ACCME System, nearly all CME activities now include outcome measurement at one or more levels. In 2024, 95% measured learner competence, 46% measured performance, 18% measured patient health, and 8% measured community or population health [1].

This evolution reflects a shift from measuring participation to measuring impact. Accreditation requirements ensure that outcomes are systematically planned, assessed, and verified. As providers employ more objective assessments, the evidence base linking CME to healthcare improvement continues to strengthen. Systematic reviews continue to affirm that CME/CPD can improve clinician performance and, under supportive organisational conditions, patient outcomes [10,11]. Interventions that include multiple components, repeated engagement, and explicit alignment with system priorities demonstrate the greatest effect

[2]. These findings reinforce the importance of integrating accredited CME/CPD within continuous quality-improvement frameworks rather than viewing it as a stand-alone educational event.

Education is the principal mechanism through which discovery becomes practice. Clinical guidelines, new therapeutics, and diagnostic technologies require structured opportunities for learning, rehearsal, and feedback before they reliably change behaviour [2]. Isolated dissemination, emails, memos, prompts in the electronic record or online postings, rarely produce sustained adoption. Accredited CME closes this gap by embedding deliberate practice, local data feedback, and peer discussion, thereby converting information into implementation [2,12].

Beyond advancing performance, CME also plays a vital role in maintaining and restoring professional competence. Remedial CME, when applied appropriately, has been shown to reduce future disciplinary actions among physicians. In a recent national study of more than 4,000 physicians disciplined by state medical boards, those required to complete remedial CME as part of their first disciplinary action were significantly less likely to experience additional discipline within five years, with an odds ratio of 0.6 [13]. These findings affirm that CME can not only advance quality and safety but also help restore professional standards, serving as a rehabilitative mechanism that protects both patients and the integrity of the profession.

Accredited CME also functions as a strategic partner for institutional change. When aligned with quality improvement and patient safety initiatives, education supports measurable progress towards organisational goals. At several large health systems, accredited CME integrated with quality improvement initiatives has led to measurable declines in central-line infections and improved hypertension control, illustrating how deliberate learning cycles create a self-reinforcing model in which education drives measurable change and outcome data refine future learning [2].

Providers recognised with commendation exemplify this integration of learning and leadership. The Menu of Criteria for Accreditation with Commendation rewards organisations that use education to achieve outcomes in learner performance, healthcare quality, and patient or community health. Between 2017 and 2022, 40% of 122 applicants achieved commendation [14]. Of these, 62% documented improvements in clinician performance, 48% in healthcare quality, and 31% in patient or community health, evidence that accreditation incentives can catalyse innovation and accelerate adoption of evidence-based practice [14].

However, CME does not produce these results by chance. Education must be deliberately designed to achieve impact. The effectiveness of CME depends on evidence-based instructional design such as using needs assessment, interactivity, deliberate practice, feedback, and assessment to ensure that learning translates into performance [2]. Skilled CME professionals understand how to create these conditions, but they must be empowered by institutional leaders to innovate, to adapt formats and pedagogy, and to align education with improvement goals [15]. When organisations support these professionals as partners in system change, CME achieves its full potential as a driver of improvement rather than a compliance exercise.

Strengthening Teams and Engagement Across Professions

High-quality care depends on effective teamwork, and accredited CME serves as a critical platform for interprofessional collaboration [3]. Systematic reviews confirm that team-based learning improves communication, understanding of roles, and patient outcomes [16]. Interprofessional continuing education (IPCE) activity is at record levels, with more than 113,000 accredited IPCE activities. Within the ACCME system, commendation criteria such as Promotes Team-based Education and Enhances Skills encourage design and measurement of education that improves collective performance.

Education does more than refine technical skill, it restores meaning and purpose in clinical work. Accredited CME reduces burnout by fostering connectedness, competence, and community [17–19]. Through longitudinal formats like clinical case conferences and interprofessional continuing education, colleagues and teams develop habits of reflection, feedback, and mutual accountability that reinforce safety and collaboration. Interprofessional collaboration also yields specific measurable workforce benefits. A systematic review of 48 IPCE interventions found consistent improvements in teamwork scores and communication quality, with pooled reductions in inpatient adverse events and patient length of stay [16]. Health systems that implement IPCE report an average 80% increase in interprofessional activities within two years of achieving Joint Accreditation, stronger engagement across professions, and improved staff retention through cultures of mutual respect and shared purpose [20].

By integrating CME with leadership priorities for culture and teamwork, organisations can build "educational homes" that connect professional development with system improvement. Education thus becomes both a vehicle for change and a source of resilience for clinicians and teams [20,21].

Accelerating Innovation and Disseminating Emerging Science

Accredited CME functions as a trusted bridge between scientific discovery and clinical application. It ensures that innovations – new diagnostics, therapies, and guidelines – are disseminated rapidly, accurately, and free from commercial bias [22,23]. By leveraging the convening power of education, institutions can align their learning agendas with strategic priorities, fostering continuous adoption of evidence-based advances across departments and professions [15].

Every major clinical advance such as the introduction of GLP-1 receptor agonists, genomic sequencing, or new sepsis bundles relies on CME infrastructure to disseminate protocols and train clinicians in real-world settings. Without such educational mechanisms, implementation lags and variability widens. CME thereby functions as the "last mile" of translation between research and patient care.

Enhancing Efficiency and Cost Effectiveness

Accredited CME contributes directly to healthcare value by improving efficiency and reducing waste. Education aligned with organisational quality goals decreases unnecessary procedures, lowers readmission rates, and improves prescribing accuracy [24]. Commendation-level providers have documented reductions in avoidable admissions and increased adherence to quality metrics that improve both care and cost outcomes [1].

These examples illustrate that CME is not only an educational intervention but also a performance improvement strategy that aligns clinical excellence with fiscal responsibility. For leaders, investing in CME represents a sustainable, evidence-based approach to improving value across the care continuum [17].

Improving Patient and Community Health

Accredited CME directly contributes to improved health outcomes. In 2024, nearly 44,000 CME activities measured patient health outcomes and 20,000 addressed community or population health [1]. These activities collectively reach several million healthcare professionals each year, embedding prevention and population-health principles into routine clinical workflows. Providers consistently report tangible benefits to health in the community such as increased vaccination and screening rates, improved chronic disease management, and reduced relapse rates [14].

These outcomes highlight how CME transforms data into action and connects learning to public health priorities. By addressing identified gaps in population health, accredited CME extends its reach beyond individual learners to produce measurable community impact and promote health equity.

Safeguarding Integrity and Public Trust

Trust underpins the credibility of medical education. The ACCME's *Standards for Integrity and Independence in Accredited Continuing Education* ensure that accredited CME remains free from commercial bias and that content is valid, balanced, and evidence-based [23]. These safeguards protect learners and the public, maintaining confidence that accredited CME is a trustworthy source of professional learning [25].

Historical examples of corporate influence in medicine, including the opioid crisis, demonstrate the need for such separation [26]. Accreditation ensures transparency and oversight, enabling CME to remain an ethical and reliable foundation for professional growth and public trust. This firewall between education and promotion preserves the legitimacy of professional learning as a public good.

These protections have helped preserve public confidence in accredited CME as an impartial source of learning, yet debate continues regarding the appropriate boundaries between education and industry. Debates about industry influence in medical education are longstanding and global. Critics argue that industry funding can narrow topics and introduce subtle bias, citing historical examples and calling for strict limits or bans [27]. Others propose "firewall" models or quality frameworks when industry is involved [28,29]. In the U.S., the ACCME Standards for Integrity and Independence require that accredited education be free of commercial bias in planning, content, and faculty control [22,30]. Over time, tightening these protections has coincided with a decline in the proportion of accredited activities that receive any commercial support (7% of activities in 2024 received any) and increased reliance on organisational and learner funding [1].

Incentivising and Rewarding Impact: The Commendation Framework

The commendation framework exemplifies how accreditation incentivises excellence. To achieve *Accreditation with Commendation*, providers must demonstrate compliance with eight of sixteen criteria,

including at least one from the Achieves Outcomes category that documents improvement in learner performance, healthcare quality, or patient or community health [31].

Analysis of 1,053 accreditation decisions between 2017 and 2022 showed that commendation applicants came from all provider types, including medical schools, hospitals, and professional societies, with high success rates among applicants [14]. The outcomes reported were substantive: improved guideline adherence, reductions in opioid prescribing, elimination of hepatitis A cases, improved surgical safety, reduced falls, and increased immunisation coverage [14,32]. These results demonstrate that accreditation incentives can transform provider behaviour and elevate educational strategy towards measurable system outcomes [14,20].

By encouraging providers to advance pedagogy, evaluation, leadership, and outcomes, commendation operationalises the same priorities emphasised throughout this JCME collection – teamwork, effectiveness, innovation, efficiency, engagement, and trust.

Conclusion: The Expanding Impact of Accredited CME

Accredited continuing medical education (CME/CPD) has matured into a learning-oriented system that supports both individual competence and organisational performance. More than 1,600 accredited organisations in the United States now deliver hundreds of thousands of educational activities each year, with continued growth in participation, interprofessional collaboration, and outcomes measurement.

Far from being a compliance requirement, accredited CME/CPD functions as a structural asset for healthcare organisations and a mechanism for translating evidence into practice. By investing in education that meets accreditation standards, leaders strengthen safety, efficiency, and engagement while maintaining independence and public trust.

Through rigorous standards, verified outcomes, and a skilled professional workforce, accredited CME/CPD enhances competence and teamwork, supports innovation, and contributes to better health outcomes. Its expansion into interprofessional and system-level initiatives underscores its role as a strategic resource for healthcare improvement. Sustaining independence, transparency, and scholarly evaluation will remain essential for ensuring the continued trust and impact of CME/CPD worldwide.

Disclosure Statement

No potential conflict of interest was reported by the author(s).

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